

Department of Orthopaedics
Discharge Summary

CR No.: anonymous	Name: anonymous	Age/Gender: 87.0 Yr/M
Admission No.: anonymous	Department Unit: Orthopaedics (Ortho Unit 1)	Consultant/Faculty: anonymous
Ward: 2 A 3 Ward	Room/Bed: 2 A 3 Orthopaedics/ 2A3 Ortho 11	Patient Category: General
Discharge Type: Normal Discharge	Father/Spouse Name: anonymous	Occupation:
Contact No.: anonymous	Address: anonymous	State/Country India
D.O.A. : 13-Mar-2023 19:45	D.O.D. : 08-Feb-2026	D.O. Print Report: 08-Feb-2026 15:43

FINAL DIAGNOSIS:

22 DAYS OLD MID SHAFT FEMUR FRACTURE(AO 32A)

OPERATION PERFORMED:

ORIF WITH FEMUR INTERLOCKING NAIL(PITKAR) SS (10X400MM) On 30-Mar-2023

SURGEONS: Dr Sudarsan Behera

ASSISTANT SURGEONS/SENIOR RESIDENTS: DR ANKIT GARG, DR BHASKAR RAI

OPERATIVE FINDINGS: Under CSE with patient in right lateral position left lower, limb painted and draped. Then 8 centimeter incision given centering over fracture site. TFL cut in line, fracture exposed and fracture margins freshened. Then 8mm reamer passed in proximal fragment and entry made retrograde. Then guide wire passed retrograde and incision given over entry site. Then guide passed introduced in distal fragment after reduction. Then PITKAR 10 X 400mm nail passed(SS PITKAR femur Intern locking nail) and fixed with 4.9 x 34 and 36. Proximal bolts and 4.9 x 46 x 50mm, distal bolts. Then all wounds washed and closed in layers. Patient shifted to post-op in stable condition.

CHIEF COMPLAINTS:

pain with visible deformity in left thigh since 6 days
swelling in left thigh since 6 days

HISTORY OF PRESENT ILLNESS: Patient was apparently alright, six days back, when he had alleged history of fall on March 23, 2023 at 4 pm at his residence in Bhilai due to acute episode of vertigo. The patient took rest for two days, following the incident. He was taken to Suraj Hospital where he was admitted for three days during which he was diagnosed with left femur fracture and primary treatment was given. The patient was then referred to pulse hospital, in view of cardiac fitness for surgery where he was declared unfit for surgery. Due to unfitness, the patient has now come to AIIMS Raipur for further management. There is an History of MI 19 years back since then he is on medication for the same. History of hypertension diagnosed, 19 years back and is on medication for the day. He was treated for MI 19 years back.

EXAMINATION:

02-Apr-2023

General Physical Examination

Local Examination

left thigh- skin is intact. No
sinus scar or fistula, erythema
absent, tenderness, present
over left mid thigh, Crepitus
Present over left mid thigh, hard
mobile mass present over mid
thigh, range of motion at left
hip- painful and restrictive,
active toe movement present,
DPA, PTA, palpable,

Type

Sensorium

Normal

98

Blood Pressure

Systolic

112

Diastolic

76

Temperature (in Fahrenheit)

Afebrile

Clubbing

No

98

INVESTIGATION DETAILS:

Requisition Date	Test Name	Parameter Name	Result	Ref Range & Unit
01-Apr-2023 10:05	COMPLETE BLOOD COUNT (CBC)	Comments on PBS	Refer Reports*	-
		Absolute Basophil Count	0.02	0.02 - 0.10 x10 ³ /μL
		Absolute Eosinophil Count	0.00	0.02 - 0.5 x10 ³ /μL
		Absolute Lymphocyte Count	0.90	1 - 3 x10 ³ /μL
		Absolute Monocyte Count	0.57	0.2 - 1.0 x10 ³ /μL
		Absolute Neutrophil Count	7.20	2 - 7 x10 ³ /μL
		Basophil	0.2	00 - 01 %
		Eosinophils	0.0	01 - 06 %
		HCT	31.7	40 - 50 %
		Hemoglobin	9.6	13 - 17 g/dl
		Lymphocytes	10.4	20 - 40 %
		MCH	20.3	27 - 32 pg
		MCHC	30.3	31.5 - 34.5 g/dL
		MCV	66.9	83 - 101 fL
		Monocytes	6.6	2 - 10 %
		Neutrophils	82.8	40 - 80 %
		Platelet Count	258	150 - 410 x10 ³ /μL
		RBC	4.74	4.5 - 5.5 x10 ⁶ /μL
		Red Cell Distribution Width	21.3	11.6 - 14.0 %

		(RDW CV)		
		Total Leukocyte Count(TLC)	8.69	4 - 10 x10 ³ /μL
Rft		Chloride	98	102 - 109 mmol/L
		Creatinine	1.00	0.9 - 1.3 mg/dl
		Potassium	4.90	3.5 - 5 mmol/L
		Sodium	132	135 - 145 mmol/L
		Urea	58	15.0 - 39.0 mg/dl
31-Mar-2023 00:54	RHEUMATOID	Rheumatoid Factor Rh	141.3	< 15 IU/mL
	FACTOR (RF)			
21-Mar-2023 17:10	COMPLETE BLOOD	Comments on PBS	Refer Reports*	-
	COUNT (CBC)			
		Absolute Basophil Count	0.01	0.02 - 0.10 x10 ³ /μL
		Absolute Eosinophil Count	0.13	0.02 - 0.5 x10 ³ /μL
		Absolute Lymphocyte Count	1.47	1 - 3 x10 ³ /μL
		Absolute Monocyte Count	0.56	0.2 - 1.0 x10 ³ /μL
		Absolute Neutrophil Count	6.82	2 - 7 x10 ³ /μL
		Basophil	0.1	00 - 01 %
		Eosinophils	1.4	01 - 06 %
		HCT	33.1	40 - 50 %
		Hemoglobin	9.7	13 - 17 g/dl
		Lymphocytes	16.4	20 - 40 %
		MCH	19.8	27 - 32 pg
		MCHC	29.3	31.5 - 34.5 g/dL
		MCV	67.4	83 - 101 fL
		Monocytes	6.2	2 - 10 %
		Neutrophils	75.9	40 - 80 %
		Platelet Count	332	150 - 410 x10 ³ /μL
		RBC	4.91	4.5 - 5.5 x10 ⁶ /μL
		Red Cell Distribution Width	21.3	11.6 - 14.0 %
		(RDW CV)		
		Total Leukocyte Count(TLC)	8.99	4 - 10 x10 ³ /μL
	Liver Fuction Test (Lft)	Albumin	2.7	4.0 - 5.0 gm/dL
		ALP	183	-
		ALT	22	7 - 41 U/L
		AST	39	12 - 38 U/L
		Direct Bilirubin	0.22	0.1 - 0.4 mg/dl
		globulin	4.8	2.0 - 3.5 gm/dL
		Indirect Bilirubin	0.33	0.20 - 0.90 mg/dl
		Total Bilirubin	0.55	0.3 - 1.3 mg/dl
		Total Protein	7.5	6.7 - 8.6 gm/dL
Rft		Chloride	95	102 - 109 mmol/L

17-Mar-2023 11:54	Protein Electrophoresis	Creatinine	0.88	0.9 - 1.3 mg/dl
		Potassium	4.72	3.5 - 5 mmol/L
		Sodium	130	135 - 145 mmol/L
		Urea	23	15.0 - 39.0 mg/dl
		textbox1	7.1	-
		textbox2	2.43	-
		textbox3	0.36	-
		Textbox4	0.76	-
		textbox5	0.81	-
		Textbox 7	2.74	-
16-Mar-2023 22:22	URINE FOR BENCE JONES PROTEINS (BJP)	Textbox 8	Not Detected	-
		Textbox 9	Not Detected	-
		Bence Jones Protein	Absent	-
		Comments	Refer Reports*	-
		Comments on PBS	Refer Reports*	-
		Absolute Basophil Count	0.01	0.02 - 0.10 x10 ³ /μL
		Absolute Eosinophil Count	0.15	0.02 - 0.5 x10 ³ /μL
		Absolute Lymphocyte Count	1.07	1 - 3 x10 ³ /μL
		Absolute Monocyte Count	0.48	0.2 - 1.0 x10 ³ /μL
		Absolute Neutrophil Count	8.23	2 - 7 x10 ³ /μL
15-Mar-2023 07:03	COMPLETE BLOOD COUNT (CBC)	Basophil	0.1	00 - 01 %
		Eosinophils	1.5	01 - 06 %
		HCT	32.6	40 - 50 %
		Hemoglobin	9.8	13 - 17 g/dl
		Lymphocytes	10.8	20 - 40 %
		MCH	20.0	27 - 32 pg
		MCHC	30.1	31.5 - 34.5 g/dL
		MCV	66.7	83 - 101 fL
		Monocytes	4.8	2 - 10 %
		Neutrophils	82.8	40 - 80 %
		Platelet Count	323	150 - 410 x10 ³ /μL
		RBC	4.89	4.5 - 5.5 x10 ⁶ /μL
		Red Cell Distribution Width (RDW CV)	19.6	11.6 - 14.0 %
		Total Leukocyte Count(TLC)	9.94	4 - 10 x10 ³ /μL
		Comments on PBS	Refer Reports*	-
		Absolute Basophil Count	0.01	0.02 - 0.10 x10 ³ /μL
		Absolute Eosinophil Count	0.06	0.02 - 0.5 x10 ³ /μL



Absolute Lymphocyte Count	1.07	1 - 3 x10 ³ /μL
Absolute Monocyte Count	0.29	0.2 - 1.0 x10 ³ /μL
Absolute Neutrophil Count	8.34	2 - 7 x10 ³ /μL
Basophil	0.1	00 - 01 %
Eosinophils	0.6	01 - 06 %
HCT	31.6	40 - 50 %
Hemoglobin	9.3	13 - 17 g/dl
Lymphocytes	11.0	20 - 40 %
MCH	19.5	27 - 32 pg
MCHC	29.4	31.5 - 34.5 g/dL
MCV	66.2	83 - 101 fL
Monocytes	3.0	2 - 10 %
Neutrophils	85.3	40 - 80 %
Platelet Count	316	150 - 410 x10 ³ /μL
RBC	4.77	4.5 - 5.5 x10 ⁶ /μL
Red Cell Distribution Width (RDW CV)	18.3	11.6 - 14.0 %
Total Leukocyte Count(TLC)	9.77	4 - 10 x10 ³ /μL
14-Mar-2023 09:13 HIV 1 and 2 Antibodies (Rapid Test)	Interpretation	Non Reactive
	Result	Refer Reports*
14-Mar-2023 09:12 BLOOD GROUPING ABO and RhD	Blood Group	B
	Note	Refer Reports*
	RH Type	Positive

***Refer Lab Reports for details.**

OTHER INVESTIGATIONS: TREATMENT GIVEN-

INJ. CEFUROXIME 1.5 GM BD
INJ. AMIKACIN 500MG BD
INJ. PCM 1 G TDS
INJ. PANTOPRAZOLE 40 MG OD
INJ. ONDANSETRON BD
INJ. DICLOFENAC 75 MG BD
INJ. TRAMADALOL 100M SOS
CAP VIT D3 60K IU/WK X 8WKS
TAB ECOSPORIN 75MG OD
TAB FOLIC ACID 5MG OD
TAB ATORVASTATIN 10MG OD
TAB LOSARTAN(50MG) HYDROCHLORTIAZIDE(12.5MG) OD
TAB MVBC OD



C-DAC: Centre for Development of Advanced Computing
C-56/1, Sector-62, Noida - 201307, Uttar Pradesh,
91-0120-3063311-13.



TAB CA2 VitD3 OD

Tab Vit C 500 mg TDS

CAP NINTEDONIB 150MG 1CAP BD

XERNA CREAM LA BD X 15 DAYS

CONDITION AT DISCHARGE: patient hemodynamically stable. POD 2 dressing done. wound healthy, no soakage,

ADVICE ON DISCHARGE:

TAB CEFUROXIME 500 MG BD X 5DAYS

TAB. ULTRACET 1 TAB BD X 7 DAYS.

TAB PANTOPRAZOLE 40 MG OD X 7DAYS

TAB ECOSPORIN 75MG OD TO BE CONTINUED

TAB SERRATIOPEPTIDASE 10MG TDS X 3 DAYS

TAB FOLIC ACID 5MG OD TO BE CONTINUED

TAB ATORVASTATIN 10MG OD TO BE CONTINUED

TAB LOSARTAN(50MG) HYDROCHLORTIAZIDE(12.5MG) OD TO BE CONTINUED

TAB MVBC OD X 15 DAYS

TAB CA2 VitD3 OD X 15 DAYS

CAP VIT D3 60K IU/WK X 8WKS

Tab Vit C 500 mg TDS X 15 DAYS

CAP NINTEDONIB 150MG 1CAP BD x continue

XERNA CREAM LA BD X 15 DAYS

walker assisted walking, active toe movement, knee hip ROM exercise, quadriceps strengthening exercises

ACTIVE TOE MOVEMENT

ANKLE ROM EXERCISES

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DISCHARGE BY : anonymous

DISCHARGE REMARKS :

FOLLOW UP ON : 14-Apr-2023

FOLLOW UP CONSULTANT / FACULTY : anonymous

FOLLOW UP ADVICE : REVIEW IN ORTHO OPD ON TUE/FRI after 1 week

Review in pulmo opd after 1 week

R/a 1 week in cardiology opd.

Summary Prepared By

anonymous

Summary Approved By

anonymous